

05.10 Introduction

The philosophy of Community Long Term Care Case Management in South Carolina is to optimize the participant's life choices and rights, to minimize threats to the participant's safety and health, and to provide a mechanism for managing home and community based alternatives to institutional care. The program allows for a range of options based on choice, role, and responsibility in the decision-making process for persons to direct their own home and community based long term care services through the assistance of a Case Manager for the services identified as needed in their Service Plan.

Community Long Term Care operates on the following program principles which support this case management philosophy:

1. To provide a mechanism for utilizing cost effective alternatives to institutional care and to provide services for those participants eligible within the constraints of affordability by maximizing the utilization of available resources.
2. To ensure services are appropriate to the needs of individual participants, are of acceptable quality, and do not duplicate other formal services.
3. To recognize and strive to honor the participant's decisions regarding the Participant Service Choice Form and services received. However, a decision to receive services, including case management cannot be based on gender, gender identity, race, age, color, religion, or national origin.
4. To ensure the participant's right to assume risk, commensurate with that person's ability and willingness to assume responsibility for the consequences of that risk.
5. To acknowledge and respect the participant's right to be treated with consideration and dignity.
6. To assist and not replace family members and other informal support systems currently involved with the participant's care.
7. To make service decisions utilizing a multi-disciplinary team approach based on comprehensive, on-going assessments which include input from the participant and/or Authorized Representatives.
8. To provide a well-defined, visible, and accessible point of entry for long-term care services.

Case management is a waiver service that is provided to all participants who participate in the CLTC program. Case management is a vital part of the long-term

care service system available to Medicaid eligible participants and ensures continued access to this system.

Participants can choose to have only case management services. In such situations, the value of case management is in the continued follow-up, coordination and monitoring of services other than CLTC waived services. The **Case Manager** will also be able to periodically evaluate any changes in the participant's service needs.

Case management includes the following five activities which are discussed in this chapter:

- Service Counseling.
- Service Planning.
- Service Coordination.
- Monitoring; and,
- Re-evaluation.

Conflict Free Case Management

The Case Management provider and its staff must be independent of the service delivery system and not a provider of services that could be incorporated into a CLTC participant's plan of care ("conflict free case management"). These services include, but are not limited to, CLTC waiver services, home health services, and hospice services. CLTC is the final decision authority regarding questions concerning conflict free case management.

05.20 Waiver Enrollment

Pre-admission review is completed by a **SCDHHS Worker** for a participant who applies for the waiver. Policy and Procedure must be followed for intake, assessment, level of care, and Medicaid eligibility.

A participant must be eligible is Medicaid financially eligible, meets level of care, completed the Participant Service Choice Form, and has an available waiver slot) to enter the waiver.

If the participant does not enroll in the waiver **within thirty (30) calendar days** of the initial assessment, a new assessment must be completed by the **SCDHHS Worker**. A new assessment must be completed in Phoenix and a new level of care determined and recorded in Phoenix. This re-assessment may be completed by phone or visit. If the applicant appears to be medically ineligible at any time while the case is pending, a visit must be made to the applicant to re-determine the level of care. It may be necessary to re-verify Medicaid eligibility, depending on the amount of time the case has been pending.

The **SCDHHS Worker** must call to confirm the participant's present location and ensure there have been no significant changes in the applicant's condition prior to the transfer conference and enrollment. The **SCDHHS Worker** should inform the applicant that the case is being transferred to the **Case Manager II, Lead Team Case Manager, Lead Team Nurse Consultant** or **Area Administrator** for enrollment. The **SCDHHS Worker** should notify the applicant that the **Case Manager II, Lead Team Case Manager, Lead Team Nurse Consultant** or **Area Administrator** will call to discuss service planning and service authorization.

The **SCDHHS Worker** and the **Case Manager II, Lead Team Case Manager, Lead Team Nurse Consultant** or **Area Administrator** must team staff the assessment information for service plan development and waiver enrollment. Waiver enrollment date is the date case is team staffed. The participant's current Medicaid financial category is reviewed by the **Case Manager II, Lead Team Case Manager, Lead Team Nurse Consultant** or **Area Administrator** at the time of the case transfer.

The enrolling **Case Manager II, Lead Team Case Manager, Lead Team Nurse Consultant** or **Area Administrator** must complete the following tasks on the date of enrollment which is the same date of the transfer conference:

- Narrate the transfer conference with the **SCDHHS Worker**.

- Change the participant status to “participating” in Phoenix.
- Send a CLTC Notification Form to the participant, and/or Authorized Representative notifying of the waiver enrollment. A CLTC Notification Form is also required to be sent to Medicaid eligibility to notify them of the enrollment.
- Notify the designated CLTC Support Staff for RSP indicator entry. The RSP entry must be completed within four (4) business days of the waiver enrollment date.

The enrolling **Case Manager II, Lead Team Case Manager, Lead Team Nurse Consultant or Area Administrator** must complete the following tasks within seven (7) business days of the enrollment date:

- Prior to enrollment: call participant, and/or Authorized Representative to involve in Service Plan development and review the services based on need.
- Contact the participant to confirm or obtain Case Management Provider Choice if the provider choice form has not been received. These choices must be documented on the Service Provider Report (Provider Choice Form) and in the narrative record in order of preference. The provider choice form must be scanned into Phoenix. This choice must be confirmed at the Initial Visit and the Provider Choice Form signed.

Note: If the participant and/or Authorized Representative does not respond with a Case Management Provider Choice within ten (10) business days from date it was mailed, a Notification Form must be mailed to the participant and/or Authorized Representative informing of the need to make a Case Management Provider Choice within the next ten (10) business days. If the choice is not received within the next ten (10) business days, the Area Administrator must be consulted. Prior to case closure for this reason, the Area Administrator will discuss with Regional CLTC Director.

- Explore community services and formal/informal supports.
- Review waiver services and offer option to make verbal choice of waiver service providers.
- Record choices(s) on the Service Provider Report generated from Phoenix, then initial and date form. Scan Service Provider Report into Phoenix Scans.

- If verbal choice cannot be made, mail the appropriate Service Provider Reports to the participant and/or Authorized Representative.
- Call participant and/or Authorized Representative within ten (10) business days from the date the provider list was mailed to secure choice(s) for each planned service.
- Key information in Phoenix identifying service plan interventions and updating other sections of Phoenix as needed.
- Team discuss and sign the Service Plan with **Case Manager II, Lead Team Case Manager, Lead Team Nurse Consultant or Area Administrator.**
- Authorize one (1) unit of service for Case Management for SCDHHS.
- Refer chosen services including on-going waiver Case Management.
- Send Physician's Order if needed for Nutritional Supplements or Private Duty Nursing.

Once a case is accepted by the Case Management Provider, team staffing must occur with the **Case Manager II, Lead Team Case Manager, Lead Team Nurse Consultant or Area Administrator** and **on-going Case Manager** within two (2) business days. If the **on-going Case Manager** does not contact the **Case Manager II, Lead Team Case Manager, Lead Team Nurse Consultant or Area Administrator** within two (2) business days for the team staffing to occur, the **Case Manager II, Lead Team Case Manager, Lead Team Nurse Consultant or Area Administrator** should proceed to the next Case Management Provider Choice by declining the referral and moving to the next referral. Documentation of this must be made in the Phoenix narrative. If a Case Management Provider choice is not made, SCDHHS will be the Case Manager until a choice is made.

The SCDHHS Worker must narrate all team staffing activities or attempts for team staffing.

05.30 Case Management Policy and Procedure

05.31 On-going Case Management

The **Case Manager** must make the **Initial Visit** to the participant **within thirty (30) calendar days** of the waiver enrollment date.

Exception: If the **Initial Visit** is due within five (5) business days of the on-going Case Management referral, the on-going **Case Manager** must complete the **Initial Visit** within ten (10) business days of accepting the referral. The late activity will not be subject to strikes or recoupment for timeliness if the visit is made within the ten (10) business days of referral acceptance.

If the participant is hospitalized at the time of the scheduled Initial Visit, the visit should be delayed until the participant is discharged. A visit is due **within seven (7) business days** of notification of discharge.

The **Case Manager** must complete the following during the **Initial Visit**.

Initial Visit:

- Visit participant at the place of residence.
- Complete documentation in the Electronic Visit Verification (EVV) by using the participant's landline phone for check-in and check-out or by using the EVV Mobile Phone Application. The participant's mobile phone must not be used for check-in and check-out.
- The EVV check-in call must be made at the beginning of the visit **AND** a check-out call must be made at the end of the visit.
- In cases where visits are made for more than one person in the same home, the Case Manager should check-in and check-out for one person at a time, not over lapping calls to EVV.
- Review assessment and add any updates or additional information needed in the assessment comment field.
- Review Service Plan with the participant and revise as needed.
- Obtain signature of the participant and/or Authorized Representative on the Service Plan Agreement. If no revisions are made, leave copy of Service Plan

with the participant and/or Authorized Representative. If revisions are necessary, mail a copy of the revised Service Plan to the participant and/or Authorized Representative following the visit.

- Notify the providers of any Service Plan changes via phone call or Conversation feature in Phoenix. The provider can access the updated Service Plan in Phoenix.
- If the initial assessment was completed by phone, obtain signed CLTC Consent Form in Phoenix.
- If the initial assessment was completed by phone, obtain signed Participant Service Choice in Phoenix.
- Review, update and complete Waiver Service and Non-Waiver Service (Referrals/Authorizations) tabs in Phoenix.
- Discuss the participant's Personal Goals. Document any newly stated Personal Goals in the Personal Goals Section in the Service Plan including the steps to achieve the Personal Goal. Personal Goals must be time specific, measurable and realistic. Review and document participant's progress toward previously stated Personal Goals. If the participant does not indicate a Personal Goal, document in the narrative checklist.
- If services are provided prior to the initial visit being completed: review Phoenix claims and Report for missed visits, exception codes, service delivery, service provided as authorized during correct dates and times, check- in and check-out from authorized phone, visits that are longer or shorter than authorized. This may be completed by viewing the Claims Section in Phoenix or by obtaining an activity report in Phoenix. Also, review Phoenix claims and Reports to monitor activities task sheets and activities observations. This may be completed by viewing Phoenix Reports then Claims. **This review may not be completed more than seven (7) business days prior to the contact.** Trends and patterns of problems rather than isolated cases should be noted.
- Verify Medicaid financial eligibility for waiver services. It is important for the Case Manager to check their dashboard often to monitor participants' Medicaid eligibility.
- Verify choice of location of services. Must obtain participant signature on CLTC Participant Service Choice form if previous choices were obtained by

verbal consent.

- Review, complete and update Participant Information section in Phoenix.
- Review, complete and update Medication Section in Phoenix.
- Review, complete and update Home Assessment Section in Phoenix.
- Review, complete and update as needed Emergency Disaster Priority Section in Home Assessment – Home Condition tab.
- In the Disaster Plan Box, record the full name, address and phone number of the individual responsible for assisting the participant in the event of an emergency/disaster.
- Review and obtain signatures on the Participant's Rights and Responsibilities Statement in Phoenix. Make every attempt to have the participant and/or Authorized Representative sign. If the Authorized Representative is unavailable, mail a copy to the Authorized Representative for signature. A copy of the Participant's Right and Responsibilities must be given to the participant and/or Authorized Representative.
- Discuss Service Provider Report and obtain provider choice(s) in order of preference for each planned service. Confirm any provider choice(s) previously obtained. Obtain participant and/or Authorized Representative's signature and date. Signed Service Provider Reports must be scanned into Phoenix Scans.

Note: If the participant is not physically or mentally able to complete and/or sign the Service Provider Report Form, the Authorized Representative may do so. The lack of a signature on the form should not delay the participant from receiving waiver services. The participant may receive waiver services as long as a written choice is indicated on the form. However, every effort must be made to secure a signature.

- Review, complete and update as needed the Caregiver Support Section in Phoenix. If no Authorized Representatives are listed, the participant should check the appropriate box on the Appointing an Authorized Representative Form 1282 LTL to verify the section was reviewed.
- Provide the CLTC Information Folder. The CLTC Information Folder must include:

- CLTC Brochure
 - Copy of Participant’s Rights and Responsibilities Form
 - Case Manager Business Card and Contact Information
 - Estate Recovery Brochure
 - Choosing a Case Manager Brochure
 - Choosing an Adult Day Health Care Brochure
 - Choosing a Personal Care I / Personal Care II Brochure
 - APS Brochure
 - Emergency Preparedness Information
 - Emergency Telephone Numbers Form
 - Evacuation Information
 - Emergency Checklist
 - Helpful Reminders in an Emergency Brochure
- During the check-out call through the Electronic Visit Verification (EVV), enter the activity completed and any new observation. Observations that represent a significant change should be entered.
 - Any follow up activities and subsequent actions required as a result of participant needs or provider compliance problem identified must be documented in the narrative record.
 - Complete the Initial Visit narrative and Narrative Checklist in Phoenix.

05.32 Service Counseling

Service counseling involves objectively consulting, discussing, advising, and listening. The CLTC **Case Manager** is in a position to confer with the participant and/or Authorized Representative concerning needs and to provide information that will assist them in making sound long term care decisions. The service counseling process includes educating the participant and/or Authorized Representative with the long term care options available to them and ensuring the participant's right to be involved in planning the care. The various service options and their expected outcomes should be clearly explored with the participant and/or Authorized Representative.

Since service counseling is not clinical in nature, it is imperative that the **Case Manager** recognize when a referral for this type of professional intervention is needed. It is essential to assure effective case management so that the **Case Manager** does not become overly involved in complex or emotional situations. The **Case Manager** must be able to distinguish his/her role in service counseling and the need for clinical intervention for the participant and/or Authorized Representative. A referral for counseling should be incorporated into the Service Plan when clinical needs are identified.

05.33 Service Planning

Person-Centered Service Planning is the active, on-going process of working together with the participant and/or Authorized Representative to assure the efficient provision of services. The **Case Manager** should constantly strive to empower the participant to become as independent as possible in advocating for his/her self and coordinating his/her own care.

Partnership with the team member and other multi-disciplinary team members, the **Case Manager** should work toward developing an agreement with the participant and/or Authorized Representative regarding the participant's needs that exist, the goals or outcomes to be reached, and the services and interventions to be explored to reach the goals.

Through person-centered service planning, the **Case Manager** consistently strives to meet the needs of the participant through the exploration of all informal and formal services.

Coordination with formal and informal services is required for the participant whose needs and/or informal supports are such that an emergency/disaster or the absence of an authorized PCII would have a substantial impact on the participant's health and safety.

Person-Centered Service Planning includes the development of a written Service Plan in partnership with the participant and parties involved in the Plan. By signing the Service Plan Agreement at the **Case Manager's** initial visit (and thereafter at the first quarterly visit following each re-evaluation), the participant, and/or Authorized Representative, confirms the Service Plan was developed with his/her input and evaluated with him/her at each re-evaluation visit.

The overall goal of person-centered service planning is to promote the health and independence of the participant while involving the participant in all phases of planning, coordination, and evaluation.

05.34 Service Coordination

Service coordination is a vital component of case management and must be documented. The **Case Manager** works together with the participant, and/or Authorized Representative and other agencies involved in the participant's care to ensure services:

- are appropriate for the participant's needs;
- meet acceptable quality standards;

- are not duplicated;
- are cost effective alternatives;
- maximize the utilization of available resources;
- are provided by other agencies in accordance with maintenance of effort agreements (Refer to Section 07.21); and,
- augment, not replace, the participant's informal support system.

05.35 **Monitoring**

The **Case Manager** monitors the plan for each waiver participant at least monthly. Monitoring should be accomplished through contacts with the participant and/or Authorized Representative. If the **Case Manager** is unable to contact the participant and/or after three (3) attempts on different days, at different times, within 10 business days, approval may be obtained and documented in the Phoenix narrative from the **Lead Team Case Manager, Lead Team Nurse Consultant or Area Administrator** to make the required contact with the Authorized Representative.

The **Case Manager** should contact formal support providers as often as the need arises. Minimally, there should be contact with in-home care providers initially and at the re-evaluation visit. These contacts must be documented in the narrative. Participant's satisfaction with and adequacy of services should be discussed with the participant, and/or Authorized Representative during the initial visit and monthly thereafter. This discussion should be included in the narrative.

All visits and monthly contacts must be conducted between the hours of 7 a.m. and 7 p.m., Monday through Friday unless the participant/Authorized Representative cannot be accommodated during this timeframe.

Note: Any exception must be prior approved and documented in the Phoenix narrative by the **Area Administrator** and/or **Central Office Representative**. Any exception must be for the benefit of the participant and not for the convenience of the **Case Manager**.

The **Case Manager** must monitor each participant monthly. At a minimum, the **Case Manager** must contact the participant via phone the first two months of the quarter and visit the participant the last month of the quarter.

The **Case Manager** must complete the following items during the **Monthly Contact**.

Monthly Contact:

- Phone the participant and/or Authorized Representative to discuss the participant's current health-impairment, appropriateness of the Service Plan and provision of formal and informal services at the level committed to. If a change in the health status does not affect the level of care or service plan, document this change in the narrative. If the change affects the service plan but not level of care, document the change in the Service Plan comments. If the change affects level of care, a re-evaluation must be completed.
- Review Phoenix Claims and Reports for missed visits, exception codes, service delivery, services provided as authorized during correct dates and times, check-in and check-out from authorized phone, visits that are longer or shorter than authorized. This may be completed by viewing the Claims Section in Phoenix or by obtaining an activity report in Phoenix. **This review may not be completed more than seven (7) business days prior to the contact.** This monitoring of claims must be completed covering the time period since the last monitoring. Trends and patterns of problems rather than isolated cases should be noticed.
- Review Phoenix Claims and Reports to monitor activities task sheets and activities observations. This may be completed by reviewing Phoenix Reports then Claims. **This review may not be completed more than (7) business days prior to the contact.**
- Verify Medicaid financial eligibility for waiver services. It is important for the Case Manager to check their dashboard often to monitor participants' Medicaid eligibility.
- Any follow up activities and subsequent actions required as a result of participant needs or provider compliance problems identified must be documented in the narrative record.
- Complete Monthly Contact Narrative including Checklist in Phoenix.

Quarterly Visit

The **Case Manager** must ensure that quarterly visits are completed timely and efficiently. The first **Quarterly Visit** is due within ninety (90) calendar days of waiver enrollment and subsequently every ninety (90) calendar days until re-evaluation, which resets the cycle.

Exception: If the **Quarterly Visit** is due within five (5) business days of the on-

going Case Management referral, the on-going Case Manager must complete the **Quarterly Visit** within ten (10) business days of accepting the referral. The late activity will not be subject to strikes or recoupment for timeliness of the visit if made within ten (10) business days of referral acceptance.

If the participant is hospitalized at the time of the scheduled quarterly visit, the visit should be delayed until the participant is discharged. A visit is due within seven (7) business days of notification of discharge and narration of the visit delay must be completed.

The **Case Manager** must complete the following items during the **Quarterly Visit**.

Quarterly Visit:

- Visit participant at place of residence.
- If a quarterly visit in a location other than the participant's residence would benefit the participant and/or Authorized Representative, the **Case Manager** must request an exception from the **Lead Team Case Manager, Lead Team Nurse Consultant, Satellite Office Supervisor** or **Area Administrator**. The approval or denial must be documented in the narrative. **The approval or denial is valid for one time.** The phone number for this alternate location must not be entered in Phoenix as an acceptable number for the Electronic Visit Verification (EVV).
- Complete documentation in the EVV by using the participant's landline phone for check-in and check-out or by using the EVV Mobile Phone Application. The participant's mobile phone must not be used for check-in and check-out.
- The EVV check-in call must be made at the beginning of the visit AND a check-out call must be made at the end of the visit.
- In cases where visits are made for more than one person in the same house, the **Case Manager** should check-in and check-out for one person at a time, not overlapping service/visit times to EVV.
- Review Assessment Information and add updates or additional information in the assessment comment field.
- Review the Person-Centered Service Plan and revise as needed.

- Obtain signature of participant and/or Authorized Representative on the Service Plan Agreement at the first quarterly visit following the Re-evaluation.
- If no revisions are made, leave a copy of Service Plan with participant and/or Authorized Representative. If revisions are necessary, mail a copy of the revised Service Plan to the participant and/or Authorized Representative following the visit.
- Notify the providers of any Service Plan changes via phone call or conversation feature in Phoenix. The provider can access the updated Service Plan in Phoenix.
- Review, update and complete Waiver Service and Non-Waiver Service (Referrals/Authorizations) tabs in Phoenix.
- Review Phoenix Claims and Reports missed visits, exception codes, service delivery, service provided as authorized during correct dates and times, check-in and check-out from authorized phone, visits that are longer or shorter than authorized. This may be completed by viewing the Claims Section in Phoenix or by obtaining an activity report in Phoenix. **This review may not be completed more than seven (7) business days prior to the contact.** This monitoring of claims must be completed covering the time period since the last monitoring. Trends and patterns of problems rather than isolated cases should be noted.
- Review Phoenix Claim and Reports to monitor activities task-sheets and activities observations. This may be completed by viewing Phoenix reports then Claims. This review may not be completed more than seven (7) business days prior to the contact.
- Verify Medicaid financial eligibility for waiver services. It is important for the Case Manager to check their dashboard often to monitor participants' Medicaid eligibility.
- Verify choice of location of services. Must obtain participant signature on CLTC Participant Service Choice form if previous choices were obtained by verbal consent.
- Review, complete and update Participant Information Section in Phoenix.

- Review, complete and update Medication Section in Phoenix.
- Review, complete and update Home Assessment Section in Phoenix.
- Review, complete and update as needed At Risk for Missed Personal Care II status in Phoenix.
- Review complete and update as needed Emergency Disaster Priority status in Home Assessment under Home Condition tab.
- Review, complete and update as needed Emergency Disaster Preparedness Section in Home Assessment under Home Condition tab. In the Disaster Plan Box, record the full name, address and phone number of the individual responsible for assisting the participant in the event of an emergency/disaster.
- Review, complete and update as needed Caregiver Support Section in Phoenix. If no Authorized Representatives are listed, the participant should select the appropriate box on the Appointing an Authorized Representative Form 1282 LTL to verify the section was reviewed.
- Discuss the participant's Personal Goals. Document any newly stated Personal Goals in the Personal Goals Section in the Service Plan including the steps to achieve the Personal Goal. Personal Goals must be time specific, measurable and realistic. Review and document participant's progress toward previously stated Personal Goals. If the participant does not indicate a Personal Goal, document in the narrative checklist.
- During the check-out call through the EVV, enter the activity completed and any new observations. Only new observations that represent a significant change should be entered.
- Any follow up activities and subsequent actions required as a result of participant needs or provider compliance problem identified must be documented in the narrative record.
- Complete **Quarterly Visit** narrative and narrative checklist in Phoenix.

Some participants may require more monitoring than others. Frequency of contacts should be determined by prioritizing participants whose medical conditions are unstable, participants who require complex service plans, and/or participants who

have limited support systems.

A participant has the right to assume risk, commensurate with his/her ability and willingness to assume responsibility for the consequences of that risk. This, of course, does not abrogate a state's statutory duty to ensure the health and welfare of individuals served under home and community based services waivers. Staff should exercise good professional judgment in dealing with cases where the participant's health, safety, and welfare are questionable. These cases should be staffed with **designated Lead Team** and/or **Area Administrator**. Central Office should also be consulted. Legal counsel may be needed. Established policy (participant choice, monitoring, participant at risk, or non-compliant participant) must be followed. Documentation is crucial in these cases. Referrals must be made to other agencies (DMH, DSS, etc.) as appropriate and/or as required by State Law.

For home environments which have been determined to be unsafe or potentially unsafe for the **Case Manager**, a monitoring strategy should be developed with the **designated Lead Team and Area Administrator** based on the **CLTC Procedure Manual Safety Policy**.

05.35.10 Monitoring for Missed PCII Visit, Emergency/Disaster Priority and Emergency/Disaster Preparedness

As part of the ongoing case monitoring process, the Case Manager must identify the participant with special needs and/or inadequate supports. The participant may require special intervention in the event of a missed PCII visit or emergency/disaster.

At-Risk for Missed Personal Care II Visit Participant

A participant at risk for a missed Personal Care II visit is one whose health and safety could be compromised if the service is not provided as scheduled. Some examples are:

- Diabetic participant who resides alone, is unable to prepare a meal, must eat at specific times and the Personal Care II is responsible for preparing the meal.
- An incontinent participant who relies solely on the Personal Care II to perform bathing and toileting.
- A bedbound participant who relies solely on the Personal Care II to perform activities of daily living and meal preparations.

If a participant is determined by the **Case Manager** to be "At Risk for a

Missed PCII visit," this information must be communicated to the provider at the time of the referral and any time the status changes. The At-Risk status for Missed Personal Care II Visit is keyed in the Participant Information section of Phoenix. This will flag appropriate Service Authorizations to alert providers of the At- Risk Personal Care II visit status. Service planning must address this special need of the participant. (Refer to Section 05.33 and 07.23)

The provider is required to contact the **Case Manager** if services cannot be provided to the participant as authorized and backup efforts have failed. The **Case Manager** must respond immediately in an attempt to secure the assistance needed by the participant. Efforts to secure assistance must be documented in the Narrative.

At a minimum, interventions will typically include:

- contacting participant and/or Authorized Representative to assess the situation; efforts should be made to involve the participant and/or Authorized Representative in identifying possible solutions;
- reviewing the Service Plan for other informal support systems which could be contacted to assist the participant; and/or
- exploring a change in the PCII provider or authorizing an alternate service on an emergency basis (i.e., ADHC).
- Participants who have been coded as “At Risk for Missed Personal Care II” in Phoenix may be identified using the Case Manager Roster Report, Personal Care II Visit Status.
- Service Planning must address this special need of the participant.

Emergency/Disaster Priority

The Emergency/Disaster Priority Section in the Home Assessment section under Home Condition tab of Phoenix must be reviewed and updated as necessary, and at least during the Initial Visit, each Quarterly Visit and Re-Evaluation Visit.

The Emergency/Disaster Priority status is entered in Phoenix according to the description below. Service planning is required in an attempt to meet the needs of a participant who would be vulnerable during an emergency/disaster.

1. Not Priority - Participant is not vulnerable during emergency/disaster or has adequate supports to meet needs. (Example: Participant has functional deficits but family is willing and able to evacuate and/or

meets needs.)

2. Priority/Lives Alone – Participant lives alone and is vulnerable in emergency/disaster due to limitation of support system. (Example: Participant lives alone and has no one available to assist with evacuation or has no one to administer medications.) If this is chosen, answer the question in Phoenix either yes or no “Participant consents to have emergency preparedness agencies notified.”
3. Priority/Advanced Medical Need – Participant has advanced medical needs and would be vulnerable during an emergency/disaster. (Example: Participant is ventilator dependent, require dialysis or other specialized equipment or services.) If this is chosen, answer the question in Phoenix either yes or no “Participant consents to have emergency preparedness agencies notified.”
4. Priority/Other – Participant has special needs as identified by the **Case Manager** and/or **Nurse Consultant**, and would be vulnerable during emergency/disaster. (Example: Participant can only be transported by ambulance or participant with special communication needs.) If this is chosen, answer the question in Phoenix either yes or no “Participant consents to have emergency preparedness agencies notified.”

Emergency /Disaster Preparedness

The Emergency/Disaster Preparedness Section in the Home Assessment section under Home Condition tab of Phoenix must be reviewed and updated as necessary, and at least during the Initial Visit, each Quarterly Visit and Re-evaluation Visit. If the participant plans to remain in the home during an emergency/disaster, the checklist for goods and services must be completed. Only goods and services which are not already available in the home and would be needed in the event of an emergency/disaster should be checked. The full name, address and telephone number of the individual responsible for assisting the participant in the event of an emergency/disaster must be included in the Disaster Plan Box.

The Pre-Disaster Checklist and Post-Disaster Checklist

The Pre-Disaster Checklist and Post-Disaster Checklist will be used to document and report Pre-Disaster and Post-Disaster efforts when an emergency or disaster is imminent and the Emergency/Disaster Plan has been implemented by Central Office or the Area Office.

05.36 Assessment Completion at Re-Evaluation

The Assessment is used to re-determine a waiver participant's long-term care needs. Information obtained during the re-evaluation process should be adequate for the

Case Manager to re-determine the participant's level of care, service planning, and service needs.

It is the Case Manager's responsibility to ensure the assessment is completed accurately and thoroughly before a level of care is determined.

05.37 Re-Evaluation at 365-Day Intervals

The **Case Manager** will schedule re-evaluation visits in a timely manner to allow completion of all re-evaluation activities within the three hundred and sixty-five (365) calendar day time frame to complete re-evaluations for all waiver participants. The re-evaluation visit due date for participants receiving case management, is scheduled by Phoenix for completion every three hundred and sixty-five (365) calendar days. The three hundred and sixty-five (365) calendar day cycle begins with the last assessment completed by the **Nurse Consultant** prior to waiver enrollment. Subsequent re-evaluations will be scheduled three hundred and sixty-five (365) calendar days from the date the last assessment was completed. The completed assessment for re-evaluation visit, levels of care, and service plans must be completed by the due date. When the re-evaluation is conducted on a date other than the due date, the three hundred and sixty-five (365) calendar day cycle will be adjusted and subsequent re-evaluations will be scheduled three hundred and sixty-five (365) calendar days from the date the last assessment was completed.

Re-evaluation Visit

The **Case Manager** must complete the Re-evaluation Visit with the participant by the three hundred and sixty-fifth (365th) calendar day from the last assessment. **All data (assessment, service plan and narrative) must be uploaded to Phoenix by the three hundred and sixty-fifth (365th) calendar day.**

NOTE: All waiver documentation for the month must be keyed, entered and uploaded to the network by 11:59 pm of the last day of each month. This supersedes any documentation timeliness and policy guidelines. For example, documentation related to re-evaluations completed the last week of the month but not due until the first week of the following month, must be completed by 11:59 pm of the month the visit was completed.

The **Case Manager** must complete the following items during the **Re-evaluation Visit**.

Re-evaluation Visit:

- Visit participant at the place of residence.
- If a re-evaluation visit in a location other than the participant's residence would benefit the participant and/or Authorized Representative, the **Case**

Manager must request an exception from the **Lead Team, Lead Team Nurse Consultant, Satellite Office Supervisor** or **Area Administrator**. The approval or denial must be documented in the narrative. The approval or denial is valid for one time. The phone number for this alternate location must not be entered in Phoenix as an accepted number for the Electronic Visit Verification (EVV).

- Complete documentation in the EVV by using the participant's landline phone for check-in and check-out or by using the EVV Mobile Phone Application. The participant's mobile phone must not be used for check-in and check-out.
- The EVV check-in call must be made at the beginning of the visit AND a check-out call must be made at the end of the visit.
- In cases where visits are made for more than one person in the same home, the Case Manager should check-in and check-out for one person at a time not overlapping calls through the EVV.
- Complete a new Assessment at re-evaluation.
- Determine the Level of Care and complete the Level of Care staffing.
- Develop a new Person-Centered Service Plan with input and participation from the participant/Authorized Representative and others if requested by the participant/Authorized Representative.
- All Re-evaluation activities must be completed by the three hundred and sixty-fifth (365th) day.
- Complete Service Coordination and narrate contacts made.
- Review, update and complete Waiver Service and Non-Waiver Service (Referrals/Authorizations) tabs in Phoenix.
- Review Phoenix Claim and Report for missed visits, exception codes, service delivery, service provided as authorized during correct dates and times, check-in and check-out from authorized phone, visits that are longer or shorter than authorized. This may be completed by viewing the Claims Section in Phoenix or by obtaining an activity report in Phoenix. This review may not be completed more than seven (7) business days prior to the contact. This monitoring of claims must be completed covering the time period since the

last monitoring. Trends and patterns of problems rather than isolated cases should be noted.

- Review Phoenix Claims and Reports to monitor activities task sheets and activities observations. This may be completed by viewing Phoenix Reports then Claims. **This review may not be completed more than seven (7) business days prior to the contact.**
- Verify Medicaid financial eligibility for waiver services. It is important for the Case Manager to check their dashboard often to monitor participants' Medicaid eligibility.
- Verify choice of location of services. Must obtain participant signature on CLTC Participant Service Choice form if previous choices were obtained by verbal consent.
- Review, complete and update Participant Information Section in Phoenix.
- Review, complete and update Home Assessment in Phoenix.
- Review, complete and update as needed At Risk for Missed Personal Care II status in Phoenix.
- Review, complete and update as needed Emergency Disaster Priority status in Home Assessment under the Home Condition tab.
- Review, complete and update as needed Emergency Disaster Preparedness Section in Home Assessment under the Home Condition tab. In the Disaster Plan Box, record the full name, address and phone number of the individual responsible for assisting the participant in the event of an emergency/disaster.
- Review, complete and update as needed Caregiver Support Section in Phoenix. If no Authorized Representatives are listed, the participant should select the appropriate box on the Appointing an Authorized Representative Form 1282 LTL to verify the section was reviewed.
- Discuss the participant's Personal Goals. Document any newly stated Personal Goals in the Personal Goals section of the Service Plan including the steps to achieve Personal Goals. Personal Goals must be time specific, measurable and realistic. Review and document participant progress towards previously stated Personal Goals. If the participant does not indicate a Personal Goal, document in the narrative checklist.

- During the check-out call through the Electronic Visit Verification (EVV), enter the activity completed and any new observations. Only new observations that represent a significant change should be entered.
- Any follow up activities and subsequent actions required as a result of participant needs or provider compliance problems identified must be documented in the narrative record.
- Complete and send any pertinent communication such as Service Authorization /Termination, Service Plan, Physician's Order or Notification Form.
- Complete Re-evaluation Visit narrative and narrative checklist in Phoenix.

05.37.10 Hospitalized Waiver Participant Who is Due an Initial Visit or Re-evaluation Visit

If a participant is hospitalized at the time of the regularly scheduled Initial Visit, Quarterly Visit or Re-evaluation Visit, the visit should be delayed until discharge. Regular case management activities, except a monthly phone contact, must be interrupted during hospitalization. This contact must be documented in the narrative. All other waiver services must be interrupted or terminated according to the professional judgment of the **Case Manager**.

The Initial Visit or Re-evaluation visit is due **within seven (7) business days** of the notification of the participant's discharge from the hospital. At this time, a complete review of the case is required. All activities required during the Initial Visit or the re-evaluation visit must be completed. The Service Plan must be completed within seven (7) business days of the assessment completion. The Initial Visit or Re- evaluation Visit must be completed with verification of the participant's locus choice and Medicaid financial eligibility. The three hundred and sixty-five (365) calendar day cycle will be adjusted and subsequent re-evaluations will be scheduled three hundred and sixty- five (365) calendar days from the date the assessment was completed following discharge.

05.37.20 Re-Evaluation with Interruption of 365-Day Cycle Due to ADL/Medical Changes

It may be necessary, due to reported or observed changes in a participant's condition, to complete a Re-evaluation before the Re- evaluation is due. This Re-evaluation must be completed at the participant's home. A re-evaluation

must include a re-determination of the participant's level of care and service needs, verification of locus choice, and verification of Medicaid financial eligibility. Contact must be made with identified agencies and/or providers to ensure service coordination. (Refer to Section 05.34.) A new Person-Centered Service Plan must be developed at each re-evaluation. A copy of the new Service Plan will be mailed to the participant and/or Authorized Representative. If the re-evaluation is completed prior to the month that it is due, the next re-evaluation will be due three hundred and sixty-five (365) calendar days from the date the re-evaluation was completed. When a re-evaluation is completed instead of another activity due, the re-evaluation narrative checklist will be completed. For example, if a Re-evaluation is completed when an Initial Visit is due, complete the Re-evaluation checklist instead of the Initial Visit checklist.

05.38 Waiver Participant Determined as Medically Ineligible

If a waiver participant appears to be medically ineligible, the **Case Manager** must complete the following tasks:

- Obtain medical information from the physician by mailing or faxing the Physician's Input Letter and the CLTC Consent Form 121.
- Re-visit participant for clarification as necessary which may include a face-to-face visit by a **SCDHHS Worker**.
- Conduct a team conference with a **SCDHHS Worker** to reach a joint level of care decision. If the assessment was completed by a SCDHHS Worker, the team conference will be completed by two SCDHHS Worker.
- If a level of care decision cannot be reached, consult with the **Area Administrator**. If the case cannot be resolved at the Area Office level or if efforts to verify medical information with physician are unsuccessful, the **Area Administrator** will forward the case to Waiver Service Exceptions (via email WES@scdhhs.gov).
- Complete the Level of Care section in Phoenix.
- Discuss Level of Care decision and possible alternatives with participant and/or Authorized Representative.
- Notify all providers of case closure via phone call or Phoenix Conversation. Case Manager is responsible for sending a CLTC Notification Form 171 to central Medicaid Eligibility to notify them of the case closure.
- Send a Client Notification Form to participant/Authorized Representative

informing them of case closure. Participant/Authorized Representative has ten (10) business days to respond from the date of the notification.

Note: Participant can continue to receive services through date of case closure.

- Document all activities in the Narrative.
- Terminate all services in Phoenix effective date of case closure.
- Email the **designated Support Staff** for RSP closure.

If the participant chooses to appeal the decision prior to the date of termination (within the ten (10) business day notice period) and notifies the Case Manager and requests continue benefits, the participant may continue to participate in the waiver pending an appeal hearing if the date of action has passed. For example, if a participant is given notification on September 5th that the case will be terminated effective September 15th and the participant appeals the decision prior to September 15th, services may remain in place pending the appeal hearing decision. If the participant files an appeal after September 15th and termination has already occurred, services will not be resumed. Services would only be resumed following the hearing, if the hearing officer's decision is in favor of the participant.

05.38.10 Financial Ineligibility

If at any point a waiver participant becomes financially ineligible for waiver services, all services must be terminated and the case must be closed effective the last date of financial eligibility.

If **Case Manager** receives notification that a waiver participant's financial eligibility is in question, **Case Manager** must complete the following tasks:

- Document verification of financial ineligibility via MMIS screen, letter from Social Security Administration, or written notice from Medicaid Eligibility.
- Notify involved agencies by telephone call of participant's termination from CLTC program.
- Contact participant/Authorized Representative to discuss termination, possible alternatives, and referral to other agencies.
- Complete a CLTC Notification Form 171 to notify participant and/or Authorized Representative indicating the date of termination will be the date of financial ineligibility. An Appeals Notice will not be included with this notification. Case Manager is responsible for sending a CLTC Notification Form 171 to central Medicaid

Eligibility to notify them of the case closure.

- Document all activities in the Narrative.
- Terminate all waiver services in Phoenix effective no later than the same date as financial ineligibility.
- Complete Status Changes section of Phoenix to terminate case as “Financially Ineligible.”

A waiver participant who loses financial eligibility for Medicaid for no more than one month due to excess resources, lump sum payment, etc., and is closed to waiver, can immediately be re-entered into the waiver after both financial and medical eligibility have been re-established and verified. A new application must be made with community locus choice and assigned to a **SCDHHS Worker**. Normal intake, assessment procedures and timeliness standards will be followed.

Note: If the case is closed for seven (7) business days or less, a problem may be reported in Phoenix to determine if the case may be re-opened without a new application.

05.40 Institutionalization of a Waiver Participant

If Medicaid sponsorship is required for nursing home placement, the individual must be referred to Centralized Intake for a nursing home placement application. The individual will have more than one application active in Phoenix, one assigned to the Case Manager and one assigned to the **SCDHHS Worker**. The **Case Manager** and **SCDHHS Worker** must coordinate the case activities and communicate regarding nursing home placement.

Institutionalization of Waiver Participant for Less than a Full Calendar Month

If a waiver participant is hospitalized or institutionalized (i.e., rehabilitation center, nursing home, DDSN/DMH facility) for a planned, temporary stay not to exceed one full calendar month, the waiver application should remain open. (Participant remains in a waiver slot and open in Phoenix.) However, case management activities must be interrupted during the period of hospitalization or institutionalization with the exception of a monthly phone contact by the assigned **Case Manager**. This contact regarding the participant's status should be documented in the narrative. All other waived services must be either interrupted or terminated (Refer to Section 07.26) according to the professional judgment of the **Case Manager**.

If the participant is discharged from the hospital or institution by the end of the last day of the full calendar month, the waiver services may be resumed. Upon discharge the **Case Manager** verifies and documents the participant's status, updates the service plan, and authorizes (or reinstates) services as appropriate. It may be necessary, due to reported changes in the participant's condition to complete a re-evaluation. This can be accomplished by completing the Assessment, re-determining the level of care, and reviewing the service needs. (Refer to Section 05.37)

Institutionalization of a Waiver Participant for More than a Full Calendar Month

Note: A full calendar month is defined as from the first instant of the month throughout the last instant of the month. For example, if a participant enters a facility before 12:00 a.m. (midnight) on the first calendar day of April and remains throughout the end of April, then waiver closure must occur April 30th. However, if the participant enters a facility after 12:00 a.m. (midnight) on April 1st or on any other calendar day in April, waiver closure must occur on May 31st.

If the participant remains hospitalized or institutionalized for a full calendar month, the waiver application must be closed to the waiver effective no later than the last day of the full calendar month period and the services, which were previously interrupted, must be terminated. This participant must be closed to the waiver as “Exceeded Full Calendar Month” in Phoenix. In order to facilitate waiver re-entry for the participant expected to return to the community, the **Case Manager** will send (at time of waiver termination) a CLTC Notification Form 171 to the participant and/or Authorized Representative, requesting that CLTC be notified of participant's impending discharge. Case Manager is responsible for sending a CLTC Notification Form 171 to central Medicaid Eligibility to notify them of the case closure.

05.41 Incarceration of a Waiver Participant

If the **Case Manager** learns that the waiver participant is incarcerated (in jail or prison), all waiver services except Case Management must be terminated immediately. The participant must be closed to the waiver by or on the last calendar day of the month. No services (including Case Management) may be billed in the following month.

05.50 Non-Compliant Participant

If a participant and/or or Authorized Representative refuses to cooperate with the CLTC Program, and all alternatives (i.e., counseling, personal contracts, referrals to DSS Adult Protective Services, interagency team staffing, case manager/supervisory visits, etc.) have been exhausted, termination of a waiver service and/or termination from the CLTC Program may be appropriate.

Examples of non-compliance include but are not limited to:

- Repeated refusal to cooperate with providers and/or **Case Manager**;
- Repeated incidences of non-compliance with the service plan;
- Physical abuse or repeated verbal abuse toward provider and/or **Case Manager**; or
- ☐ Conduct which adversely impacts the program's ability to ensure service provision or to ensure the participant's health or the safety of the CLTC representative.

If **termination of a waiver service for non-compliance** is being considered, the following must occur:

- All efforts to work with the participant and/or Authorized Representative must be documented;
- Termination of service must have prior approval of **Area Administrator or Lead Team Designee** and documented in the Phoenix narrative;
- A CLTC Notification form must be approved by The Regional CLTC Director and documented in the Phoenix narrative prior to mailing;
- Participant must be notified of termination of waiver services on the CLTC Notification; and,
- The Service Provision Form terminating service(s) must be sent to the provider(s).

If **termination from the waiver for non-compliance** is being considered, the following must occur:

- The participant and/or Authorized Representative must be notified in writing by certified mail and regular (First Class) US mail of the program requirements, advised of potential consequences of continued non-compliance and given an opportunity to remedy the circumstances. The letter must have **approval** from the Regional CLTC

Director prior to sending to the participant;

- All efforts to work with the participant and/or Authorized Representative must be carefully documented; and,
- Termination must be prior approved and documented by the designated Lead Team member, Area Administrator and Regional CLTC Director.

05.60 Leave of Absence from the State/CLTC Region of a Waiver Participant

Individuals enrolled in Medicaid home and community-based waivers who travel out-of-state may retain a waiver slot under the following conditions:

1. The trip out-of-state is a planned, temporary stay, not to exceed ninety (90) consecutive calendar days which are authorized prior to departure; and
2. The Case Manager receives notification and approves the service prior to the travel.
3. The individual continues to receive a waiver service; and,
4. Waiver services are limited to the frequency of services currently approved in the participant's plan of service (No additional hours will be authorized for vacation); and,
5. Waiver services must be rendered by South Carolina Medicaid providers and,

Note: If the Personal Care II or attendant normally uses the participant's landline in the home for Electronic Visit Verification (EVV) check-in and check-out, and the travel is prior approved by the Case manager then a cell phone is allowable to use for check-in and check-out for the service.

6. The individual must remain Medicaid eligible in the State of South Carolina.

The required waiver monitoring must continue to ensure the participant's health and welfare.

If a participant is in another state temporarily, waiver services must be interrupted or terminated, according to the professional judgment of the **Case Manager**, with the exception of Case Management and possibly PCII or Attendant Care. Documentation of the participant's status and verification of the participant's intent to return should be completed during the monthly phone contact.

If a participant is out-of-CLTC region or out-of-state at the time of a regularly scheduled re-evaluation or for an initial visit, the re-evaluation or initial visit should be completed early if possible but maybe delayed until the participant returns. If delayed the re-evaluation or initial visit must be completed **within seven (7) business days** of when CLTC is made aware of the participant's return to the CLTC region or state. If the participant continues to remain out-of-state after ninety (90) calendar days have elapsed, the case must be closed to the waiver and all waiver services terminated.

If a participant is in another CLTC region temporarily, the assigned **Case Manager** may negotiate with the involved service provider(s) to coordinate continued services. **Regular case management activities, including (at a minimum) a monthly phone contact by the assigned Case Manager and documentation of the participant's status, should continue.** Verification of the participant's intent to return should also be made during the monthly contact. If the participant remains in another CLTC region for **more than ninety (90) calendar days**, the case will be transferred to that CLTC office or closed to the waiver and all waiver services terminated.

05.70 Interactions with Hospice

A participant **can** be in a waiver and be enrolled in the Hospice Benefit at the same time but is limited in the services that may be authorized. (See Hospice Benefits Interaction Section.) If it is learned that a participant has been receiving services not allowed on the Hospice Benefits Interaction Table, the service(s) must be terminated immediately using the calendar day the information was received as the termination date. For example: Participant admitted to Hospice on 04/15/2016 and on-going **Case Manager** notified on 04/30/2016 of hospice admission. Personal Care II, Personal Care I, briefs, med pads and adult wipes are terminated effective 04/30/2016 to coincide with the date the **Case Manager** was notified of the hospice admission.

A participant, who has been admitted to a Hospice House, may receive Case Management service but no other waiver services. All monitoring activities continue as scheduled, including visits. The waiver application does not need to be closed for “Exceeds Full Calendar Month”.

Note: Palliative Care is not considered to be Hospice Care.

05.80 Process for Critical Incidents

A **Critical Incident** is any actual or alleged event or situation that creates a significant risk of substantial or serious harm to the physical or mental health, safety or well-being of a SCDHHS waiver participant, Home Again participant or PRIME participant.

State workers or Provider Case Managers have a duty to report critical incidents if they have reason to believe that a vulnerable adult or child is being abused, exploited or neglected. **State worker or Provider Case Managers must submit the critical incident in Phoenix within 24 hours or the next business day after discovery of the suspected or alleged abuse, neglect or exploitation.**

SCDHHS also monitors for other reportable incidents. These are defined as incidents involving death or serious harm to a beneficiary/participant. Reportable incidents include:

- Abuse (intentionally inflicting or allowing to be inflicted physical injury on a vulnerable adult or child by an act or failure to act. Deliberately subjecting a vulnerable adult or child to threats or harassment or other forms of intimidating behavior. Abuse can be physical, verbal, psychological or sexual)
- Neglect (the failure or omission of a caregiver to provide the care, goods, or services necessary to maintain the health or safety of a vulnerable adult or child)
- Exploitation (causing or requiring a vulnerable adult or child to engage in activity or labor which is against the reasonable and rational wishes of the vulnerable adult or child. Includes the improper, unlawful, or unauthorized use of funds, assets, property, power of attorney, guardianship, or conservatorship of a vulnerable adult or child by someone for the profit or advantage of that person or another person)
- Unexplained deaths
- Falls (resulting in death, injury requiring hospitalization, injury that will result in permanent loss of function)
- Traumatic injuries (e.g., third degree burns over more than ten (10%) percent of the body that result in death, require hospitalization, or result in a loss of function)
- Unauthorized restraints (both chemical and physical, that results in death, hospitalization, or loss of function in an institutional setting only)

- Media related events (any report that presents a potential or harmful characterization of the Department of Health and Human Services or any of its contracted entities that involves a SCDHHS waiver participant, Home Again participant or PRIME participant)
- Infectious disease outbreak
- Elopement (unexplained absence for more than 24 hours)

Note: If the **CLTC Case Manager/Nurse Consultant** is unsuccessful in collaborating consistently with the APS Supervisor or the APS Case Manager, the **Case Manager or Nurse Consultant** will report the problem to the **Area Administrator** or to the **Lead Team** who will contact the APS Supervisor or the APS Case Manager. If this is not successful, the **Area Administrator or Lead Team** should contact the Regional APS Consultant and if this is not successful, the **Area Administrator or Lead Team** should contact **SCDHHS Quality Assurance Team** for assistance.

Process for Recording a Critical Incident:

- State worker or Provider Case Manager will pull the participant record and on the carousel and select “Critical Incidents”
- Select “New Critical Incident”
- Select Program – Only programs with active applications for the participant will show
- Select Critical Incident Type - Only one type can be chosen, see list below
- Incident Date – Date that the occurrence happened, not date that it is being reported
- Description – Mandatory. Provide details of the incident and all involved parties along with their relationship to the participant
- Source of Information: Source name, Relationship to participant and phone number

Note: The Confidential box should be selected when the critical incident identifies and reports on a Provider Case Manager.

Tracking of Critical Incidents by State Workers and Provider Case Managers:

- Abuse, Neglect, Exploitation – at a minimum, monthly update from APS. To continue until APS provides a closure date or notifies of a report not accepted after 45 calendar day investigation

- Death, Falls, Traumatic injuries and Unauthorized Restraint – Single entry, no additional tracking required
- Media-related events and Elopement – at a minimum, weekly update until resolved

There will not be a notification in Phoenix that monitoring is due. SCDHHS Quality Assurance will contact the State Worker or Provider Case Manager if monitoring becomes delinquent.

Critical Incident Types:

- Abuse
- Neglect
- Exploitation
- Fall Resulting in Death
- Fall Resulting in Hospitalization
- Fall Resulting in Permanent Loss of Function
- Traumatic Injury Resulting in Death
- Traumatic Injury Resulting in Hospitalization
- Traumatic Injury Resulting in Permanent Loss of Function
- Unauthorized Physical/Chemical Restraint Resulting in Death
- Unauthorized Physical/Chemical Restraint Resulting in Hospitalization
- Unauthorized Physical/Chemical Restraint Resulting in Permanent Loss of Function
- Unexplained Death
- Elopement (24 hours or More)
- Media-Related Event Resulting in Harmful/Potentially Harmful Portrayal of SC DHHS and/or a Contracted Entity of SC DHHS
- Infectious-Disease Outbreak
- Pressure Ulcers That Are Unstageable or Staged III & IV (PRIME ONLY)

- Emergency Room Visit (HOME AGAIN ONLY)
- Death Following Break-Down in 24 Hour Back-Up System (HOME AGAIN ONLY)
- Involvement with Criminal Justice System (HOME AGAIN ONLY)
- Medication Administration Error (HOME AGAIN ONLY)

05.90 Process for Visits to Participants with Active Bedbug Infestation

When bedbug infestation is suspected and an Enhanced Pest Control provider has scheduled but not made an initial visit to assess the home for bedbugs or when bedbug infestation has been confirmed by an Enhanced Pest Control provider but treatment has not been completed, and the **Case Manager** does not want to enter the home, the **Case Manager** must:

- Request permission from the **Area Administrator** or **Lead Team** to complete the Quarterly Visit or Re-evaluation Visit outside the participant's home.
- Receive approval or denial from the **Area Administrator** or **Lead Team**. The **Area Administrator** or **Lead Team** must document the request and decision in the narrative.
- Observe the participant directly while completing the visit on the front porch, or deck of the participant's home, ADHC (if participant attends ADHC), at the **Area Office** or other public place.
- Use the EVV mobile application to check-in and check-out for the visit.
- Add comment in Service Plan, Home Assessment section and Narrative regarding confirmed bedbug infestation and treatment.
- Make the next scheduled visit inside the participant's home after the treatment has been completed.
- If the participant is not eligible for the Enhanced Pest Control service or refuses treatment, the **Case Manager** must consult with the **Area Administrator** for guidance.